Memorial General Hospital - Department of Inpatient Medicine

CLINICAL DOCUMENTATION — NARRATIVE INPATIENT DISCHARGE SUMMARY V1

Patient: Michael Brown | **MRN:** PAT-60005 | **DOB:** 1969-07-30 | **Gender:** Male | **Doc ID:** NARRATIVE_0005
Encounter Date: 2025-03-05 | **Discharge/Note Date:** 2025-03-08 | **Provider:** Dr. Emily Chen, MD

CHIEF COMPLAINT

The patient presented with generalized fatigue, productive cough, and persistent fevers.

HISTORY OF PRESENT ILLNESS

The patient is a 64-year-old male with a medical history pertinent for hypertension and dyslipidemia who presented for evaluation of generalized fatigue, productive cough, and persistent fevers. Symptoms began approximately four days prior to admission and escalated in severity. On presentation to the emergency department, initial diagnostic evaluation demonstrated that white blood cell count was 7.2 10*3/uL. Concurrently, serum chemistry was notable for a total cholesterol of 114.7 mg/dL (LOW). Vital signs upon arrival were stable, and the patient was admitted to the inpatient general medicine service.

HOSPITAL COURSE

During the hospital course, the inpatient team initiated targeted management. The patient received Metformin 500 mg PO BID with good clinical tolerance. Serial assessments revealed gradual resolution of presenting symptoms. Prior to planned discharge, repeat laboratory testing confirmed that bun stabilized at 4.22 mg/dL (LOW). The patient was ambulating without assistance and tolerating a regular diet.

DISCHARGE MEDICATIONS

The patient was discharged home in stable clinical condition with instructions to take Amlodipine 2.5 mg PO once daily, and Lisinopril 5 mg PO every morning. Medication reconciliation was completed and all questions regarding dosing schedules were answered.

DISCHARGE INSTRUCTIONS & FOLLOW-UP

The patient was advised to follow up with their primary care physician within 7 to 10 days. Return precautions for chest pain, recurrent shortness of breath, or fever were provided and understood.